

FEVER

A patient with a fever has a temperature $\geq 38^{\circ}\text{C}$ now or in past 3 days.

Give urgent attention to the patient with a fever and any of:

- Fits or just had a fit $\hookrightarrow$ 19.
- Decreased consciousness $\hookrightarrow$ 16
- Neck stiffness, drowsy/confused or purple/red rash, **meningitis** likely
- Respiratory rate > 30 or difficulty breathing
- BP $< 90/60$
- Tender in right lower abdomen, **appendicitis** likely
- Severe abdominal or back pain
- Jaundice
- Easy bleeding or bruising

Management:

- If BP $< 90/60$, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90 . Continue 1L 6 hourly. Stop if breathing worsens.
- If likely **meningitis**, decreased consciousness, fits or respiratory rate > 30 /difficulty breathing: give **ceftriaxone** 2g IV¹/IM. Avoid injecting $> 1\text{g}$ IM at one injection site.
- If patient was in a malaria area in past 3 months and malaria test² positive: give **artesunate** 2.4mg/kg IM and notify. Refer urgently within 6 hours. Record artesunate dose in referral letter. If artesunate unavailable, give quinine as slow IV infusion over 4 hours: dilute **quinine** 20mg/kg in **dextrose 5%** 5-10mL/kg. If IV not possible, give IM³ diluted in **sodium chloride 0.9%**.
- If glucose < 3 or $\geq 11.1 \hookrightarrow$ 17.
- Refer urgently.

Approach to the patient with a fever not needing urgent attention

- If on abacavir, check for abacavir hypersensitivity reaction (AHR) $\hookrightarrow$ 116.
- Has patient been in a malaria area in past 3 months?

Yes: arrange same day malaria test². If not available same day, refer.

No

Malaria test positive

Malaria likely

- Notify and give **artemether/lumefantrine** 80/480mg with food/milk: immediately, then after 8 hours, then 12 hourly for 2 days (total of 6 doses). If patient vomits within the 1st hour of taking treatment, give the same dose again.
- Also consider other cause of fever (see adjacent).
- Check Hb and glucose.
- Give urgent attention and refer same day if: Hb < 7 , glucose < 3 , unable to take orally or symptoms worsen.
- Refer same day if: > 65 years old, pregnant, known HIV/diabetes or malaria treatment not available.

Malaria test negative

Consider other cause of fever:

Does patient have a tick bite (small dark brown/black scab) or tick present?

Yes

Tick bite fever likely:

- May also have headache, body pain, rash or localised lymphadenopathy.
- If tick present, grip tick close to skin using forceps and remove.
- Give **doxycycline** 100mg 12 hourly for 7 days. If pregnant, give instead **azithromycin** 500mg 12 hourly for 3 days.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for 5 days.
- If severe headache or no better after 3 days, refer.

No

- Ask about other symptoms: assess and manage on symptom page.
- **Acute viral infection** likely (such as influenza or COVID-19) if any of: cough, sore throat, loss of taste or smell, nose symptoms, headache, body pain. If the current prevalence of COVID-19 is high, consider COVID-19 $\hookrightarrow$ 40.

If none of above:

- Check urine dipstick: if blood, leucocytes or nitrites $\rightarrow$ 59.
- Exclude TB $\hookrightarrow$ 92.
- Test for HIV $\hookrightarrow$ 110.
- Advise patient to return if other symptoms develop.
- If previous malaria test negative and fever persists after 2 days, repeat malaria test².
- If fever persists for > 5 days and cause still uncertain, discuss/refer.

¹Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ²Test for malaria with rapid diagnostic test if available, and parasite slide microscopy. ³To give IM quinine: first calculate volume of sodium chloride 0.9% in mL: weight $\times 20 \div 100$. Then add this volume of sodium chloride 0.9% to quinine 20mg/kg and inject half the volume into each thigh.